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CUSTOMER	IDAMAN PHARMA MANUFACTURING SDN BHD
DESCRIPTION	Cloxacilla Powder For Syrup
MEASUREMENT	W:143MM X H: 143MM
MATERIAL	SMPO 050

COLOUR	: BLACK
FINISHING	: FOLDING

MATERIAL CODE:

Lt-604.10

Attn :	MS. SITI
From :	MAHA
I hereby verify & confirm that this draft is the final copy. Any substantial changes to the design and contents of the end product will not be borne by THUNDER PRINT SDN. BHD.	
Customer Signature & Date	

21.7.2022

IDAMAN PHARMA MANUFACTURING

CLOXACILLA POWDER FOR SYRUP 125MG/5ML

DESCRIPTION

A slightly pink to pale red powder which is an orange syrup with an orange and pineapple odour on reconstitution.

COMPOSITION

Each 5 ml of the reconstituted suspension contains CLOXACILLIN SODIUM equivalent to 125 mg CLOXACILLIN.

PHARMACODYNAMICS

Cloxacillin is a bactericidal antibiotic which inhibits bacterial cell wall synthesis, probably by acylation of membrane-bound transpeptidase enzymes. This prevents cross-linkage of peptidoglycan chains which is necessary for bacterial cell wall strength and rigidity. Also, cell division and growth are inhibited and lysis of susceptible bacteria frequently occurs. Rapidly dividing bacteria are most susceptible to penicillin group of antibiotics. Cloxacillin is resistant to staphylococcal penicillinase. It has an antibacterial spectrum similar to that of methicillin.

PHARMACOKINETICS

Cloxacilla is well absorbed; administration of higher doses result in a proportionately higher increase in serum levels. Concurrent administration of probenecid to patient with normal renal function causes a higher and prolonged serum level. Cloxacillin is excreted by glomerular filtration and renal tubular secretion.

INDICATIONS

Cloxacilla is used for infections due to Staphylococci resistant to benzylpenicillin. It is also used for mixed Streptococcal and Staphylococcal infections when the Staphylococci are penicillin-resistant.

DOSAGE AND ADMINISTRATIONS

Cloxacilla is effective in a wide range of dosage regimens. The recommendation given here are commonly applied.

Route	Administration	Dosage
Children (oral)		
Up to 2 years	½ - 1 hour before meal	62.5 - 125 mg every 6 hours
2 - 10 years	½ - 1 hour before meal	125 - 250 mg every 6 hours

In severe infections the doses may be safely increased. The prescribed course should be completed.

DIRECTION FOR MIXING DRY SYRUP

Add a little water and shake till powder is dispersed. Slowly add water up to the mark on the bottle/label and shake well.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Cloxacilla is contraindicated in patients with history of allergic reactions to penicillin or cephalosporins.

PRECAUTIONS/ WARNINGS

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Cloxacilla Powder for Syrup 125mg/5mL, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactams agents. If an allergic reaction occurs, Cloxacilla Powder for Syrup 125mg/5mL must be discontinued immediately and appropriate alternative therapy instituted.

Prescribing this medication in the absence of a proven or strongly suspected bacterial infection is unlikely to provide benefit to the patient and risks the development of drug-resistant bacteria.

Candidiasis and other superinfections may occur, especially in debilitated and malnourished patients, or those with low resistance to infection due to corticosteroids, immunosuppressive agents or irradiation. If superinfection occurs, institute appropriate measures.

During long-term therapy, renal, hepatic and hematopoietic functions should be checked periodically.

The passage of any penicillin from blood into brain is facilitated by inflamed meninges and during cardiopulmonary bypass. In the presence of such factors, particularly in renal failure when high serum concentrations can be attained, central nervous system adverse effects including myoclonia, convulsive seizures and depressed consciousness can be expected. Although this complication has not been reported with cloxacillin, it should be anticipated.

Special precaution should be given to patients with asthma and history of seizure disorder.

Experience in premature and newborn infants is limited. Cautious administration of the drug to such patients and frequent evaluation of organ system function is recommended.

This product containing aspartame, hence it is unsuitable for phenylketonurics.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

This medicine may affect your ability to drive or use machines. If you feel sick, dizzy or tired, or have headache, do not drive or use machines and contact your doctor immediately.

INTERACTION WITH OTHER MEDICATIONS

Cloxacillin sodium has been reported to be incompatible with aminoglycosides, tetracyclines, and other antimicrobial agents including erythromycin, polymyxin B, pantoprazole, hydromorphone, chloramphenicol, sulphonamides.

Contraceptives oestrogen containing oral

Concurrent use with cloxacillin may decrease the effectiveness of oral contraceptive because stimulations of estrogen metabolism of reduction in enterohepatic circulation of oestrogen, resulting in menstrual irregularities, intermenstrual bleeding and unplanned pregnancies.

Probenecid

Decrease renal tubular secretion of penicillins when used concurrently. This effect results an increased and prolonged serum concentrations, prolonged elimination half-life and increased risk of toxicity.

Other oral anticoagulants such as anisindione and dicumarol

Penicillins may occasionally potentiate the risk of bleeding in patients treated

with oral anticoagulants, especially in the elderly and patients with uremia or hepatic impairment. The INR should be checked frequently and anticoagulant dosage adjusted accordingly. Patients should be advised to promptly report any sign of bleeding to their doctors.

BCG, typhoid vaccine and cholera vaccine

May diminish the effect of BCG, typhoid vaccine and cholera vaccine. BCG should generally be postponed until completion of antibiotic therapy. Live, attenuated oral typhoid vaccine should not be administered during and for at least 3 days before and after treatment with antibacterial sulfonamides or other antibiotics. Live, attenuated cholera vaccine should not be administered during or for at least 14 days after treatment with systemic antibiotics.

Methotrexate

May increase the risk of methotrexate toxicity.

Sodium picosulfate

May diminish the therapeutic effect of sodium picosulfate.

Mycophenolate

May decrease serum concentrations of mycophenolate. Close clinical and laboratory monitoring for evidence of diminished immunosuppressive effect of mycophenolic acid is recommended.

Entecavir

Co-administration with entecavir may result in increased plasma concentrations of one or both drugs due to competitive inhibition of transporters in the renal tubules. Patients should be monitored for excessive pharmacologic effects of both drugs, and the dosages adjusted as necessary.

Pemetrexed

Co-administration with pemetrexed may delay and/or decrease the clearance of pemetrexed. The mechanism is competitive inhibition of the renal excretion of pemetrexed, which is primarily eliminated unchanged via glomerular filtration and active tubular secretion. Patients should be monitored for excessive pharmacologic effects of both drugs, and the dosages adjusted as necessary. The potential for increased toxicity such as bone marrow suppression should be considered.

Guar gum

May decrease the plasma levels of orally administered penicillins by reducing their absorption. Separating the times of administration by as much as possible if these agents must be used together.

Proguanil

May delay and decrease gastrointestinal absorption of cloxacillin. These drugs should probably be administered at least 1-2 hours apart.

Sodium iodide I-123 and I-131

May affect the pharmacokinetics, pharmacodynamics, and/or diagnostic results of radioiodides. Co-administration of these drugs should generally be withheld for 1 week.

PREGNANCY AND LACTATION

Cloxacillin crosses the placenta and is distributed into breast milk. It should be used with caution in neonates, pregnancy and lactation.

ADVERSE EFFECTS

As with other penicillins, adverse effects are usually mild and transitory in nature. This may include skin rashes, nausea, vomiting and diarrhea, fever, joints pains and angioedema.

Others include:

Hypersensitivity reactions

Eosinophilia (with or without overt allergic manifestations), pruritus, serum sickness-like reaction with fever, chills, myalgia. Anaphylaxis and allergic reactions (rash, urticaria) including wheezing and sneezing, have occasionally been encountered.

Hematologic effects

Eosinophilia, leucopenia, anemia, thrombocytopenia, thrombocytopenic purpura, neutropenia and agranulocytosis have been reported during therapy with penicillins. These reactions are usually reversible on discontinuation of therapy and are believed to be hypersensitivity phenomena. Mild elevated SGOT levels have been reported.

Renal effects

Acute interstitial nephritis, azotemia.

Hepatic Affects

Intrahepatic cholestasis.

Gastrointestinal effects

Gastrointestinal disturbances, such as nausea, vomiting, epigastric discomfort, flatulence and loose stools have been noted in some patients.

STATEMENT FOR OVERDOSAGE AND TREATMENT

Symptoms: Extended effects of the adverse reactions.

Treatment: Supportive

If allergic reactions occur, the drug should be discontinued.

Since there is no specific antidote, treatment of penicillin overdose should be symptomatic and supportive. Haemodialysis may aid in the removal of penicillins from the blood. Serious anaphylactoid reactions require immediate emergency treatment.

STORAGE CONDITION

Store below 30°C, away from direct light.

Powder for dry syrup, need no refrigeration until reconstituted.

Once reconstituted, refrigerate and use within 7 days.

SHELF LIFE

Cloxacilla Powder For Syrup 125mg/5ml

3 years from the date of manufacture

PRESENTATION

Cloxacilla Powder For Syrup 125mg/5ml

(Malaysia Reg. No.: MAL06111328AZ)

i. Packed in HDPE bottles of 60mL and 100mL in the Pouch.

Date of Revision: 19th July 2022

PRODUCT REGISTRATION HOLDER/ MANUFACTURER:

IDAMAN PHARMA MANUFACTURING SDN BHD (200401023395)

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